

SUBJECTIVE:

The patient reports experiencing chest pain and is requesting recommendations for medicine to alleviate it. The onset, character, radiation, severity, and duration of the pain are currently unknown. No specific aggravating or alleviating factors have been identified. Associated symptoms such as shortness of breath, palpitations, dizziness, nausea, vomiting, or diaphoresis are not specified. The patient's medical history, current medications, allergies, and social history (e.g., smoking, alcohol use, illicit drug use) are not available. Family history of cardiac disease is also unknown.

OBJECTIVE:

Expected physical exam findings in a patient with acute chest pain may include signs of distress such as diaphoresis, pallor, or tachypnea.

Cardiovascular: Heart rate may be tachycardic or bradycardic, rhythm potentially irregular. Heart sounds might reveal new murmurs, gallops (S3, S4), or friction rubs. Peripheral pulses should be assessed for symmetry and quality.

Pulmonary: Lungs may be clear to auscultation, or crackles might be present if heart failure is developing. Respiratory rate may be elevated.

Abdominal: Soft, non-tender, non-distended.

Extremities: Absence of edema, warm to touch.

Vital signs are currently unavailable. Critical vital signs for this presentation would include blood pressure, heart rate, respiratory rate, temperature, and oxygen saturation.

ASSESSMENT:

Primary diagnosis: Acute Chest Pain, etiology pending. This is a critical symptom that requires immediate investigation to rule out life-threatening conditions.

Differential diagnosis:

- * Cardiovascular: Acute Coronary Syndrome (Unstable Angina, NSTEMI, STEMI), Pericarditis, Myocarditis, Aortic Dissection, Pulmonary Embolism.

- * Pulmonary: Pneumothorax, Pneumonia, Pleurisy.

- * Gastrointestinal: Gastroesophageal Reflux Disease (GERD), Esophageal Spasm, Peptic Ulcer Disease.

- * Musculoskeletal: Costochondritis, muscle strain.

- * Psychological: Panic Attack.

PLAN:

Tests:

- * Immediate 12-lead Electrocardiogram (ECG) to assess for ischemic changes or other cardiac abnormalities.

- * Cardiac biomarkers (Troponin I or T) to detect myocardial injury.

- * Chest X-ray to evaluate for pulmonary pathology (e.g., pneumonia, pneumothorax) or cardiac enlargement.

- * Complete Blood Count (CBC), Basic Metabolic Panel (BMP), and Coagulation studies.

- * Consider D-dimer if pulmonary embolism is suspected.

Medications (Acute Management):

- * Aspirin (chewable, 162-325 mg) if no contraindications, especially if ACS is suspected.

- * Nitroglycerin (sublingual 0.4 mg) if blood pressure is stable, to relieve pain and assess for anginal response.

- * Oxygen therapy if hypoxic (O2 saturation less than 90%).

- * Pain management (e.g., Morphine) if pain is refractory to nitroglycerin.

Patient advice:

- * Emphasize that chest pain is a serious symptom requiring urgent medical evaluation in an emergency department.

- * Advise against self-medicating or delaying professional assessment.

- * Educate on warning signs of a heart attack: crushing chest pain, pain radiating to the arm/jaw/back, shortness of breath, sweating, nausea.

Follow-up instructions:

- * The patient must proceed immediately to the nearest emergency department for comprehensive evaluation and management. This is not a symptom that can be safely managed with remote advice or delayed follow-up.